

Pre-Authorization/Prior Authorization Request Form

Complete all Sections to ensure timely review

If service(s) are related to Cancer, Specialty Medications or Behavioral Health please use the designated form

***All preauthorization requests must be submitted with supporting clinical documentation that is relevant to the request.**

Forms will be returned if not filled out accordingly or if they are submitted without the required clinical information.

Fax to [redacted] or for Urgent Services fax to [redacted]

Provider appeals submitted on this form will not be considered. Please use the claim resubmission request form found on our website.

Section A: Request Information

Today's Date: 08/29/2022

Completed by: [redacted]

Decisions on preauthorization requests submitted with all necessary clinical information will be made within 15 calendar days of receipt of the request.

☒ Service is Scheduled (only if applicable)

Schedule Date: 10/21/2022

☐ Urgent Request (only if applicable)

Reason for Urgency: _____

Decisions on urgent requests submitted with all necessary clinical information will be made within 72 hours of receipt of the request. Urgent requests should not be selected to accommodate a schedule date less than 15 days out unless it meets ERISA guidelines, which state the following:

According to ERISA, urgent is defined as "any claim for medical care or treatment with respect to which the application of the time periods for making non-urgent care determinations: 1. Could seriously jeopardize the life or health of the claimant or the ability of the claimant to regain maximum function or; 2. In the opinion of a physician with knowledge of the claimant's medical condition, would subject the claimant to severe pain that cannot be adequately managed without the care of treatment that is the subject of the claim".

☐ Retro Request (Decisions for retro requests, which refers to services already rendered and denied as member liability, take up to 30 calendar days)

Section B: Type of Request (check appropriate box)

Inpatient Admission: ☐ New Admission ☐ Concurrent Admission ☐ Skilled Nursing ☐ Inpatient Rehabilitation

Outpatient Service:

- | | | |
|---|---|---|
| ☐ Whole Body Imaging | ☐ Advanced Imaging of Neck and Spine, MRA, and PET | |
| ☐ Breast Procedures | ☐ Cardiac Nuclear Medicine | ☐ Cochlear Implant |
| ☐ Gastric Neurostimulator | ☐ Genetic Testing | ☐ Vein Surgery |
| ☐ Home Health Services | ☐ Invasive Spine Procedures | ☐ Blepharoplasty/Ptosis Repair |
| ☒ Orthopedic Procedures | ☐ In Lab Sleep Study | ☐ TENS Neurostimulator |
| ☐ DME (include cost of item): _____ | | ☐ Other |
| ☐ Will Neuromonitoring be used during this surgery? If so, by what provider: _____ | | |

Section C: Member Information ALL INFORMATION REQUIRED

Member Last Name: [redacted] First Name: [redacted] Date of Birth: [redacted] 1961

Subscriber #: [redacted] Phone: [redacted]

Section D: Service Information ALL INFORMATION REQUIRED (If related to Cancer or Behavioral Health use the designated form)

Description of Service: RIGHT CARPAL TUNNEL RELEASE, RIGHT CARPOMETACARPAL ARTHROPLASTY

Procedure Code (CPT/HCPCS): 64721,25447 Diagnosis Code: M18.11,G56.01

Service Start Date: 10/21/2022 Units: 1 EACH Service Frequency: 1 DAY

Section E: Facility and Servicing Provider Information ALL INFORMATION REQUIRED

Facility Name: [redacted]

Servicing Provider: [redacted]

Location: [redacted]

Location: [redacted]

Facility NPI (Required): [redacted]

Provider NPI (Required): [redacted]

Phone: [redacted]

Phone: [redacted]

Fax: [redacted]

Fax: [redacted]

NOTE: A release of information form included in the application for insurance was signed by our member.

Please note that the preauthorization of any procedure does not guarantee benefits or payment. Approval is based on medical necessity as defined in the patient's benefit plan or certificate. All benefits are subject to the term, conditions and exclusions of the benefit plan or certificate. This may include policy language regarding pre-existing conditions or signed affidavits stating that the insurance bears no responsibility, as signed by the insured. Policy exclusions for certain types of services may also apply. For additional benefit information, please contact Health Tradition at 844.825.9319.

1961

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08/31/2022 - Office Visit: L CTR, CMC, ORIF 12 days PO rm 15

Provider: MD

Location of Care:

Coronavirus Exposure

Have you traveled outside the United States in the last 14 days? No

Have you been in contact with anyone under investigation with or a confirmed case of coronavirus? No

Do you have any of the following symptoms (fever, cough, short of breath)? No

Visit Type: Follow-up

Referring Provider: MD

Primary Provider: MD,

Chief Complaint: Lt hand

Dominant Hand: Right Hand

History of Present Illness:

The patient returns to the office today for concern of pin migration. The patient has now receded to below skin level. It is not productive of much pain. There are no signs of infection currently.

Review of Systems

MS

Complains of swelling of joints.

Physical Exam

Musculoskeletal:

On focused physical examination the patient soft tissues are healing appropriately without signs of infection. X-rays of the left hand do not demonstrate any signs of lysis about the Kirschner wire. There is no clinical rotational deformity of the index finger.

Impression & Recommendations:

Assessment:

I recommended no intervention at this time aside from a short course of oral Keflex and follow-up as previously scheduled. The patient is amenable to this plan and willing to proceed in this fashion.

Patient Name:

DOB: 1961 Age: 61 Years Old

Vital Signs

Height: 61 inches **Weight:** 141 pounds
Body Mass Index: 26.74 **Body Surface Area (m2):** 1.63

Previous Medications:

- 1) rabeprazole 20 mg tablet, delayed release (dr/ec) (rabeprazole) take 1 tablet by mouth once daily
- 2) buspirone 15 mg tablet (buspirone) take 1 tablet by mouth 2 times daily for anxiety
- 3) bupropion hcl 150 mg tablet sustained-release 12 hr (bupropion hcl) take 1 tablet by mouth 2 times daily for depression and anxiety
- 4) lorazepam 0.5 mg tablet (lorazepam) take 2 tablet by mouth 3 times daily as needed for anxiety
- 5) pregabalin 100 mg capsule (pregabalin) take 1 capsule (100 mg total) by mouth three times daily.
- 6) escitalopram oxalate 20 mg tablet (escitalopram oxalate) take 1.5 tablets (30 mg total) by mouth every day.
- 7) oxcarbazepine 150 mg tablet (oxcarbazepine) take 3 tablets (450 mg total) by mouth twice daily.

Changed Medications:

- 1) Added new medication of cephalexin 500 mg tablet (cephalexin) Take 1 tablet by mouth four times a day ; Route: BY MOUTH; Stop Date: 09/05/2022 - Signed
- 2) Rx of cephalexin 500 mg tablet (cephalexin) Take 1 tablet by mouth four times a day ; #20 tablet x 0; Signed; Entered by: [REDACTED] RN; Authorized by: [REDACTED] MD; Method used: [REDACTED] Ph: [REDACTED]

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Allergies:

- 1) * GABAPENTIN (Critical)

Problems:

- 1) Overweight (ICD-278.02) (ICD10-E66.3)
- 2) Body Mass Index 26.0-26.9, adult (ICD-V85.22) (ICD10-Z68.26)
- 3) Osteoarthritis of right first CMC joint (ICD-715.14) (ICD10-M18.11)
- 4) Carpal Tunnel Syndrome, Right (ICD-354.0) (ICD10-G56.01)
- 5) Unspecified fracture of second metacarpal bone, left hand, initial encounter for closed fracture (ICD10-S62.301A)
- 6) Carpal Tunnel Syndrome, Left (ICD-354.0) (ICD10-G56.02)
- 7) Osteoarthritis of left first CMC joint (ICD-715.14) (ICD10-M18.12)

Medications, allergies, and problems were reviewed at this visit.

Past Medical History:

Reviewed history from 08/11/2022 and no changes required:

Arthritis
GERD

Past Surgical History:

Reviewed and updated today:

R breast 2018
hysterectomy
R knee scope- meniscus
Bil feet
hernia
appendix
L CMC Nov 2021 OSI
Left hand hardware removal April 2022 OSI
8/19/22 L CMC arthroplasty, ORIF 2nd MC fx Dr

Family History Summary:

Reviewed history Last on 08/02/2022 and no changes required:09/08/2022
First Degree Blood Relative - Has No Known Family History - Entered On: 9/24/2021

Social History:

Reviewed history from 09/24/2021 and no changes required:

Occupation:
Marital Status: Married-Spouse Name:

Smoking History:
Patient currently smokes every day.

Risk Factors:

Smoked Tobacco Use: Current every day smoker
Smokeless Tobacco Use: Never
Counseled to Quit/Cut Down: yes

Dietary Counseling: yes

Alcohol Use: yes

Drug Use: no

Previous Tobacco Use: Signed On - 08/02/2022

Smoked Tobacco Use: Current every day smoker

Smokeless Tobacco Use: Never

Counseled to Quit/Cut Down: yes

No Dietary Counseling Reason: Yes - Overweight

Alcohol Use: yes

Drug Use: no

Orders Added:

1) SNOMED-CT: 30346009 Established Patient Encounter for Quality Reporting [SCT-30346009]

]The written text associated with this medical record and documentation was completed utilizing voice recognition software. As a result and despite that editing was performed, there may exist inadvertent grammatical and spelling errors associated to deficits in the dictation/voice- recognition software.

Electronically signed by [REDACTED] MD on 09/08/2022 at 7:00 AM
